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SHANGHAI JUNSHI BIOSCIENCES CO., LTD.*
上海君實生物醫藥科技股份有限公司
(a joint stock company incorporated in the People's Republic of China with limited liability)
(Stock code: 1877)

VOLUNTARY ANNOUNCEMENT – APPROVAL FOR MARKETING OF TORIPALIMAB IN HONG KONG SAR, CHINA

This announcement is made by Shanghai Junshi Biosciences Co., Ltd.* (上海君實生物醫藥科技股份有限公司) (the "**Company**") on a voluntary basis. Reference is also made to the overseas regulatory announcement of the Company dated 15 October 2024.

The board (the "**Board**") of directors (the "**Directors**") of the Company is pleased to announce that Topalliance Biosciences Hong Kong Limited, a wholly-owned subsidiary of the Company, has been granted a Certificate of Drug/Product Registration issued by the Pharmacy and Poisons Board of Hong Kong (the "**PPB**"), certifying that the new drug application (the "**NDA**") for toripalimab (trade name in Hong Kong SAR, China: LOQTORZI®) in combination with cisplatin/gemcitabine, for the first-line treatment of adult patients with metastatic or recurrent locally advanced nasopharyngeal carcinoma ("**NPC**"), and for toripalimab, as a single agent, for the treatment of adult patients with recurrent, unresectable, or metastatic NPC with disease progression on or after platinum-containing chemotherapy has been approved by the PPB, making toripalimab the first and only immunotherapy drug for NPC in Hong Kong SAR, China.

ABOUT TORIPALIMAB

Drug name: Toripalimab Injection

Trade name in Hong Kong SAR, China: LOQTORZI®

Registration No.: HK-68405

Indications: Toripalimab in combination with cisplatin/gemcitabine, for the first-line treatment of adult patients with metastatic or recurrent locally advanced NPC, and toripalimab, as a single agent, for the treatment of adult patients with recurrent, unresectable, or metastatic NPC with disease progression on or after platinum-containing chemotherapy.

NPC is a malignant tumor that occurs in the epithelium of the nasopharynx. According to GLOBOCAN 2022 statistics, the number of newly diagnosed NPC cases in 2022 exceeded 120,000 worldwide. Due to the location of the primary tumor, surgery is rarely an option, while radiotherapy alone or in combination with chemotherapy are the main treatment options for localized cancers.

The approval of the NPC indications is primarily based on the results from JUPITER-02 study (a randomized, double-blind, placebo-controlled, multinational multi-center Phase III clinical study, NCT03581786, for the first-line treatment of NPC), as well as results from POLARIS-02 study (a multi-center, open-label, pivotal Phase II clinical study for second-line or later treatment of recurrent or metastatic NPC, NCT02915432).

The JUPITER-02 study is the first international multi-center, double-blind, randomized controlled Phase III clinical study in the field of immunotherapy for NPC with the largest sample size, and is the world's first Phase III clinical study in which there is preset statistical verification (Type I error control) for overall survival ("**OS**") for the first-line immunotherapy combined with chemotherapy for NPC compared to chemotherapy alone and demonstrated a survival benefit. The results of the study were presented in an oral report during the Plenary Session of the 2021 annual meeting of the American Society of Clinical Oncology (ASCO) (#LBA2), and were subsequently featured on the cover of *Nature Medicine* (IF: 58.7). The results were also published in full in the *Journal of the American Medical Association (JAMA,* IF: 63.1). The results of the study showed that, compared to chemotherapy alone, toripalimab in combination with chemotherapy reduced the risk of disease progression by 48% and the risk of death by 37%. The median progression-free survival ("**PFS**") in the toripalimab in combination with chemotherapy group was prolonged by 13.2 months compared to chemotherapy alone, from 8.2 months to 21.4 months. In addition, patients treated with this combined therapy achieved a higher objective response rate ("**ORR**") and longer duration of response ("**DoR**"), and no new safety signal was identified. Long-term survival follow-up data was presented at ASCO 2024, with a 5-year survival rate of 52% for toripalimab treatment group.

The results of POLARIS-02 study were published online in the *Journal of Clinical Oncology* (IF: 42.1) in January 2021, which showed that toripalimab demonstrates durable antitumor activity and a manageable safety profile in patients with recurrent/metastatic NPC who had failed prior chemotherapy, with an ORR of 20.5%, a DoR of 12.8 months, and a median OS of 17.4 months.

Toripalimab injection is the first domestic anti-PD-1 monoclonal antibody approved for marketing in China, and has won the "Chinese Patent Gold Award (中國專利金獎)", the top award in China's patent field. Over forty company-sponsored toripalimab clinical studies covering more than fifteen indications have been conducted globally, including in China, the United States, Southeast Asia, and Europe. Ongoing or completed pivotal clinical studies evaluating the safety and efficacy of toripalimab cover a broad range of tumor types. As at the date of this announcement, there are ten approved indications for toripalimab in Chinese mainland. In December 2020, toripalimab injection was successfully negotiated into the National Reimbursement Drug List (the "**NRDL**") for the first time. At present, six approved indications have been included in the NRDL (2023 Edition). Toripalimab is the only anti-PD-1 monoclonal antibody included in the NRDL for the treatment of melanoma. In October 2024, the PPB approved the NDA of toripalimab for the treatment of recurrent/metastatic NPC.

In terms of international layout, toripalimab had been approved for marketing as the first nasopharyngeal cancer drug in the United States and India, and had been approved for marketing by the European Commission (EC) for the first-line treatment of advanced NPC and esophageal squamous cell carcinoma ("**ESCC**"). In addition, the U.K. Medicines and Healthcare products Regulatory Agency (MHRA) accepted the marketing authorization application (MAA) for toripalimab in combination with cisplatin and gemcitabine for the first-line treatment of patients with locally recurrent or metastatic NPC, and toripalimab in combination with paclitaxel and cisplatin for the first-line treatment of patients with unresectable locally advanced/recurrent or metastatic ESCC. The Australia Therapeutic Goods Administration (TGA) and the Singapore Health Sciences Authority (HSA) accepted the new chemical entity application and the NDA for toripalimab in combination with cisplatin/gemcitabine, for the first-line treatment of adult patients with metastatic or recurrent locally advanced NPC, and for toripalimab, as a single agent, for the treatment of adult patients with recurrent, unresectable, or metastatic NPC with disease progression on or after platinum-containing chemotherapy, respectively.

IMPACT ON THE COMPANY

Upon being approved, toripalimab has become the first and only immunotherapy drug for NPC in Hong Kong SAR, China, which is conducive to the Company in further expanding the commercialization layout of its products and enhancing the international influence of its products, and is expected to have a positive impact on the Company's long-term operating results.

RISK WARNING

As pharmaceutical products are characterised by high technology, high risk and high added-value, and there is uncertainty since the commercialization of the drug is susceptible to various factors, including local policies and change in market environment, investors are advised to make cautious decisions and pay careful attention to investment risks. The Company will actively pursue the above projects and fulfill its information disclosure obligations in a timely manner for subsequent progress in strict compliance with relevant regulations.

By order of the Board
Shanghai Junshi Biosciences Co., Ltd.*
Mr. Xiong Jun
Chairman

Shanghai, the PRC, 15 October 2024

As at the date of this announcement, the Board of Directors of the Company comprises Mr. Xiong Jun, Dr. Li Ning, Dr. Zou Jianjun, Mr. Li Cong, Mr. Zhang Zhuobing, Dr. Yao Sheng, Dr. Wang Gang and Dr. Li Xin as executive Directors; Mr. Tang Yi as non-executive Director; and Mr. Zhang Chun, Dr. Feng Xiaoyuan, Dr. Meng Anming, Dr. Shen Jingkang and Dr. Yang Yue as independent non-executive Directors.

* *For identification purpose only*